

إسم المريض رابعياً : الفت عبد الحميد عثمان عبدالحليم

الحي / المدينة : بولاق الدكرور

المحافظة : الجيزة

رقم البطاقة : 29102122104024 رقم التلفون : 01156387479

العمر(تاريخ الميلاد) : 12-02-1991 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادة سرطان الثدي العلاج بعقار (Capecitabine 500mg) ما بعد الجراحة لحالات (TNBC) [14148]

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

--

Name: الفت عبد الحميد عثمان عبدالحليم National ID: 29102122104024	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المريضة تعاني من ورم غير حميد بالثدي و تحتاج الي Adjuvant capecitabine for triple-negative breast cancer
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Dms
أشياء فغناطيس على المصغ بالصف بالمقارن





Patient Name : الأستاذة / الفات عبد الحميد عثمان
Patient No. : 29102122104024
Path No. : 4082/23
Sex : Female
Age : 32 Y
Referring Doc. :

Request No. : 13004138
Request Date : 18/11/2023
Verification Date : 02/12/2023
Financial Category : اورام دار السلام - ميلانزه
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass
Nature of Specimen: Tru-cut biopsy

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 29% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 29% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :

Dr. Heba Hasan

Printing Date : 16/12/2023 09:33:40

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name الأستاذة / الفتى عبدالحميد عثمان
Patient No. 29102122104024
Path No. 4082/23
Sex Female
Age 32 Y
Referring Doc.

Request No. 13004138
Request Date 18/11/2023
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Financial Category اورام دار السلام - مبادره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar
Dr. Heba Hasan

Verified by :
Dr. Heba Hasan

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Name : OLFAT ABD AL HAMIED OSMAN

Branch : Portable

Ref.Physician Prof : CT CAP+C

Date : 22-02-2026 11:16

Medical No. : GRI-44066

Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical Data:

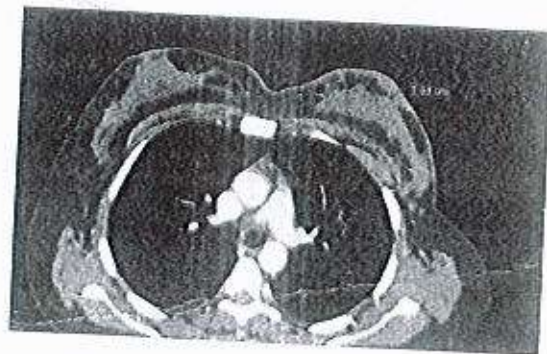
- *Known case of left breast cancer, compared to the previous study report dated 16/11/2025, the current study revealed:*

Findings:

- Stationary course is noted at left breast with skin thickening (10 mm).
- No suspicious breast lesions bilaterally.... for sono-mammography correlation.
- No suspicious axillary LNs only bilateral subcentimetric LNs are seen.
- No enlarged hilar lymph nodes
- No sizable pulmonary nodules, masses or areas of consolidation.
- Normal CT appearance of the mediastinal vessels.
- Patent tracheo-bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

OPINION:

- *Stationary course of the disease process with no denovo lesions.*



Name : OLFAT ABD AL HAMIED OSMAN

Branch : Portable

Ref.Physician Prof : CT CAP+C

Date : 22-02-2026 11:16

Medical No. : GRI-44066

Ins.Company :

**MULTI-SLICE POST CONTRAST CT SCAN OF THE ABDOMEN
AND PELVIS: -**

Findings:

- The liver is of average size (12.5 cm) with homogenous CT texture. No sizable focal lesions.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no focal lesions seen. Tiny hilar splenule is noted reaching 6 mm.
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Smooth outline of the urinary bladder yet with no definite masses or stones.
- No ascites.
- Cholecystectomy clips.
- IUD is still noted.
- Bilateral adnexal cysts, the largest seen measuring 2.5x2.5 cm on left side.
- Still noted multiple sclerotic lesions seen at both humeral heads, multiple vertebral bodies of D5 , D9 as well as left femoral head...for bone scan assessment.



Opinion:

- Stationary course of the disease process with no interval changes .
- No newly developed lesions.

Patient's Name: olfat abdel hameed othman
Date: 2026-04-09
Referred by: Prof.Dr. brain+c
Hospital: Goustave Roussey International Egypt

MRI EXAMINATION OF THE BRAIN

Technique:

- Axial T1, T2, FLAIR
- Coronal T2
- Sagittal T1
- Axial DWI and ADC
- Post contrast axial, sagittal and coronal T1

Findings:

- Few intra-axial S.O.L.'s are seen scattered in left cerebral hemisphere seen in the left frontal and parietal regions. They display hypointense signal on T1 and hyperintense signal on T2 and FLAIR and showing homogeneous enhancement. Most of these lesions are surrounded by mild perifocal edema.
- Normal cortical sulci, basal cisterns and extra-axial CSF spaces.
- Normal size and configuration of the supra-tentorial ventricular system with no midline shift.
- No evidence of blood degradation products.

Opinion:

- ☐ Few intra-axial SOL as described ?? metastatic.

for comparison please

as pt is known met.

Dr. Asser Ahmed

Breast Ca to brain

& in this, comparison is needed

much appreciated

MDT FORM 2/4/8/2020 6

Name	الفت عبا حميد عشار	MRN	C.CY / E.CY
Age	34 yrs	ID	ca. ci. ci. s. ce.
Menopausal status	Premenopausal	Phone	110734649
NAS SCORE		Marital status	married
BMI		Family history	-ve
CO morbidity	-ve	REASON OF MDT	MBC to bone New Delop Brain

Clinical Data

BX:				R		L	
ER:	PR:	HER T:	KITV:				
0/8	0/8	+5	29				
Sonomammography:							
Mets Workup: CT CAP: 19/7 SD				TNM			
BS: 25/6/2025 → no convincing evidence of Metastatic disease				T _{1c} N _{3c} M ₁ bone			
Recommendations: Premenopausal. Denovo mbc to bone. Finished 27wk paclitaxel / Carbo / Herceptin Bayde. BS 12/2024 suspicious DVG → MRI small - patient kept on Maintenance PRH Herceptin Fast on 21/6 Co seer headache - MRI → Brain Metastasis the patient sent for WBCT 17/7 - New CT CAP SD For MDT - BS → PR							
MDT OUTCOME & PLAN							

for Tykerb + Xeloda X3 Ms then re-assess

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name : Olfat Abdulhamid Uthman Abdulhamid
Branch : Maadi
Ref.Physician Prof : Hrml Hospital

Date : 25-06-2025 03:30
Medical No. : 136546
Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 34-year-old lady, with history of left breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints, more prominent at right sternoclavicular region as well as DV9, for further radiological correlation.

Impression:

- No convincing evidence of metastatic bone disease.

*Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine*

جمهورية مصر العربية
 بطاقة تحقيق الشخصية

الفت
 عبدالحميد عثمان عبدالحليم
 قرية مزغونه
 مركز البدرشين - الجيزة

١٩٩١/٠٢/١٢ ٢٩١٠٢١٢٢١٠٤٠٢٤
 JY4699780

٢٠٢٣/٠٤ ٢٩١٠٢١٢٢١٠٤٠٢٤
 بدون عمل
 متروجة
 مقيمة
 أنثى
 محمد سيد عثمان سيد
 البطاقة سارية حتى ٢٠٣٠/٠٤/٠٤

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